

11 — Valuation & Dilution

All assumptions, rates, scenarios, equity structures, and phase definitions in this document are sourced from [params.md](#). Refer to that document for any parameter clarification.

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1. Executive Summary

Health Hub is a pre-revenue health technology startup with a functional prototype, a defined target market (East Africa, Ethiopia first), and a 60/20/20 equity structure (founder / founding team + strategic partners / investor pool). Valuing a company at this stage is inherently imprecise, but the exercise is necessary for two reasons: (a) setting negotiation anchors for investor conversations, and (b) ensuring founder control is preserved through Production launch.

This document presents valuation estimates at three investment stages — Post-Pilot 1, Post-Pilot 2, and Pre-Production — using four established methodologies. It then models dilution across all relevant scenarios from params.md, demonstrating that the founder retains majority control (>50%) in every modeled outcome. Finally, it benchmarks Health Hub against comparable East African health tech funding rounds to validate that the proposed valuations are market-consistent.

Key findings:

- Pre-money valuation ranges from \$300K-\$600K (Post-Pilot 1) to \$1.5M-\$3M (Pre-Production)
- At the maximum investor pool ceiling of 20%, investor capital requirements range from \$75K-\$150K (Post-Pilot 1) to \$375K-\$750K (Pre-Production)
- The founder retains 50-60% ownership in all modeled scenarios through Production
- Comparable East African health tech seed rounds validate the \$500K-\$2M valuation corridor for companies at similar stages

2. Valuation Methodology

2.1 Methods Overview

Four valuation methods are relevant for Health Hub at different stages. No single method is definitive for a pre-revenue startup; the recommended approach is to triangulate across methods and use the convergence zone as the negotiation range.

Method	Description	Best Stage	Limitations
Comparable Transactions	Uses recent funding rounds of similar companies (geography, sector, stage) to derive implied valuations	All stages	East African health tech deal data is sparse; comparisons require adjustment for geography and maturity
Scorecard Method	Scores the startup across 7 factors (team, market, product, competition, marketing, need for additional investment, other) relative to an average seed-stage valuation	Post-Pilot 1, Post-Pilot 2	Subjective scoring; depends on accurate regional baseline average
Berkus Method	Assigns dollar values to 5 risk dimensions (sound idea, prototype, quality team, strategic relationships, product rollout/sales) up to a pre-revenue ceiling	Post-Pilot 1	Capped at ~\$2M pre-revenue; less useful after early revenue appears
Discounted Cash Flow (DCF)	Projects future free cash flows and discounts them to present value at a risk-adjusted rate	Pre-Production and later	Requires revenue data; discount rates for East African startups are very high (40-60%), making outputs highly sensitive to assumptions

2.2 Recommended Method by Stage

Investment Stage	Primary Method	Secondary Method	Validation Method
Post-Pilot 1	Scorecard	Berkus	Comparable Transactions
Post-Pilot 2	Comparable Transactions	Scorecard	DCF (sensitivity only)
Pre-Production	DCF	Comparable Transactions	Scorecard (sanity check)

2.3 Scorecard Method — Detailed Application

The scorecard method compares Health Hub to the "average" seed-stage startup in East Africa and assigns percentage weights to each factor.

Regional baseline: The average pre-seed / seed health tech valuation in East Africa is approximately \$300K-\$500K (based on available deal data from 2022-2025). We use \$400K as the baseline.

Factor	Weight	Health Hub Assessment	Score Multiplier	Weighted Contribution
Strength of team	30%	Part-time but experienced; 12 contributors; medical advisor. Below average for dedication, above average for breadth.	0.90x	27.0%
Size of opportunity	25%	\$300M-\$480M TAM across Ethiopia + Kenya. Large.	1.20x	30.0%

Product / technology	15%	Functional prototype with 10 feature modules, verified codebase (\$33K-\$45K replacement value). Strong for stage.	1.30x	19.5%
Competitive environment	10%	Fragmented; no dominant digital health platform in Ethiopia. Moderate competition in Kenya.	1.10x	11.0%
Marketing / distribution channels	5%	Admin-assisted model is unique but unproven at scale. Partner clinic channel identified but not activated.	0.85x	4.3%
Need for additional investment	5%	Company will need follow-on capital; single-round risk is moderate.	0.90x	4.5%
Other factors (regulatory, FX, market timing)	10%	Nascent regulation in Ethiopia (opportunity and risk); FX depreciation risk; strong macro tailwinds for health digitization.	0.95x	9.5%
Total	100%			105.8%

Scorecard valuation: \$400K x 1.058 = **\$423K** (at Post-Pilot 1 stage)

2.4 Berkus Method — Detailed Application

The Berkus method assigns value to five risk-reduction milestones, each worth up to \$400K for a maximum pre-revenue valuation of \$2M.

Risk Element	Assessment	Assigned Value	Rationale
Sound idea (addresses real need)	Healthcare access gap in East Africa is well-documented; \$28/year per capita health spend in Ethiopia signals massive unmet demand	\$300K	Strong need, validated by market data
Prototype exists	Functional 10-module prototype with verified codebase; 19 audit issues resolved; Angular 21 + Express 5 + PostgreSQL stack	\$350K	Beyond wireframe; working software with real architecture
Quality management team	3 founders (part-time), 7 developers, 2 advisors; medical domain expertise present; technical head with architecture skills	\$200K	Discounted for part-time commitment; no previous exit experience documented
Strategic relationships	Advisory board in place; partner clinic channel identified in Addis Ababa (2,000-3,000 private clinics); Ethio Telecom/Telebirr ecosystem access	\$150K	Relationships identified but not yet converted to binding agreements
Product rollout / sales	Pre-revenue; pilot plan defined but not yet executed; no paying customers	\$50K	Minimal credit until first external users

Berkus valuation: \$1,050K (ceiling for pre-revenue)

Note: The Berkus method produces a higher figure than scorecard because it heavily weights the existing prototype. For Post-Pilot 1 negotiation, the scorecard figure (\$423K) is more conservative and credible. The Berkus figure (\$1.05M) represents an upper bound that becomes realistic after Pilot 2 traction.

2.5 DCF Method — Framework for Later Stages

DCF is not recommended as the primary method until Post-Pilot 2 at the earliest, because pre-revenue cash flow projections carry extreme uncertainty. However, the framework is documented here for use at the Pre-Production stage.

Parameter	Value	Source / Rationale
Projection period	5 years from Production launch	Standard VC horizon
Revenue Year 1 (Production)	\$36K-\$180K/year	params.md: \$3,000-\$15,000/mo
Revenue growth rate	80-120% Y1-Y2, 50-80% Y2-Y3, 30-50% Y3-Y5	East African health tech growth benchmarks
Gross margin	60-70%	Software platform with employed clinical staff
Operating expenses (% of revenue)	70-90% Y1, declining to 50-60% by Y5	Heavy early investment in ops staff (per params.md Section 4)
Terminal growth rate	5%	Conservative for emerging market
Discount rate	45-55%	Reflects: pre-revenue stage, single-geography, FX risk, regulatory uncertainty, part-time team
Terminal value method	Exit multiple (5-8x revenue)	Comparable to health tech exit multiples in Africa

Illustrative DCF output (at Pre-Production stage):

Revenue Scenario	5-Year Cumulative FCF	Terminal Value (6x Rev)	Enterprise Value (50% discount)	Implied Pre-Money
Conservative (\$36K Y1, 50% growth)	-\$85K to +\$40K	\$440K	\$180K-\$280K	\$800K-\$1.2M
Base (\$100K Y1, 80% growth)	+\$30K to +\$180K	\$1.9M	\$600K-\$1.0M	\$1.5M-\$2.5M
Optimistic (\$180K Y1, 100% growth)	+\$120K to +\$400K	\$5.8M	\$1.5M-\$2.8M	\$2.5M-\$4.0M

The wide ranges confirm why DCF should be secondary to comparables and scorecard until real revenue data narrows the projections.

3. Pre-Money Valuation Estimates

3.1 Valuation by Investment Stage

Each stage reflects increasing de-risking: more product maturity, more traction, and more validated assumptions.

Stage	Timing (per params.md)	Traction at Entry	Valuation Range	Midpoint	Primary Method
Post-Pilot 1	Month 5-9 (after first external pilot)	Up to 1,000 users; prototype proven with real patients; partner clinics onboarded; \$0-500/mo revenue	\$300K-\$600K	\$450K	Scorecard + Berkus
Post-Pilot 2	Month 8-22 (after scaled pilot)	5,000-10,000 users; real revenue (\$500-\$3,000/mo); Kenya prep underway; staffing ladder active	\$800K-\$1.5M	\$1.15M	Comparable Transactions + Scorecard
Pre-Production	Month 12-34 (scaling to full)	Full pricing active; \$3,000-\$15,000/mo revenue; multi-geography; compliance achieved	\$1.5M-\$3.0M	\$2.25M	DCF + Comparable Transactions

3.2 Valuation Drivers at Each Stage

Driver	Post-Pilot 1	Post-Pilot 2	Pre-Production
Product maturity	Prototype proven; Android APK core ready	Hardened platform; full role workflows	Production-ready; compliance complete
User traction	Up to 1,000 (mostly subsidized)	5,000-10,000 (some paying)	10,000+ (monetized)
Revenue	\$0-\$500/mo (symbolic)	\$500-\$3,000/mo (real)	\$3,000-\$15,000/mo (scaling)
Team commitment	Part-time; no full-time employees	Part-time + employed ops staff (4-8 people)	Growing ops team (14-22 people)
Market validation	Ethiopia pilot only	Ethiopia proven, Kenya in progress	Multi-geography live
IP / technology	Codebase (\$33K-\$45K replacement)	Codebase + operational playbooks + data	Full platform + data + processes
Risk profile	High (unproven market fit)	Medium (proven demand, scaling risk)	Lower (proven model, execution risk)

3.3 Valuation Sensitivity

The following table shows how the Post-Pilot 1 valuation shifts based on key milestone outcomes.

Milestone Outcome	Valuation Impact	Adjusted Range
Pilot 1 exceeds 1,000 users	+20-30%	\$360K-\$780K
Pilot 1 generates >\$500/mo revenue	+15-25%	\$345K-\$750K
Signed clinic partnership agreement	+10-20%	\$330K-\$720K
Android APK live on Play Store	+10-15%	\$330K-\$690K
Team member converts to full-time	+10-20%	\$330K-\$720K
Pilot 1 fails to reach 500 users	-20-30%	\$210K-\$480K
No revenue by end of Pilot 1	-10-15%	\$255K-\$540K
Key team member departure	-15-25%	\$225K-\$510K

3.4 Valuation Crosswalk Between Stages

Transition	Valuation Increase	Implied Step-Up	What Justifies the Step-Up
Post-Pilot 1 to Post-Pilot 2	\$450K to \$1.15M	2.6x	Real users, real revenue, proven demand, ops team hired
Post-Pilot 2 to Pre-Production	\$1.15M to \$2.25M	2.0x	Multi-geography, full pricing, compliance, scaling ops
Post-Pilot 1 to Pre-Production	\$450K to \$2.25M	5.0x	Full journey from prototype to production-ready

4. Dilution Analysis

4.1 Starting Cap Table (Month 0)

Per params.md Section 8, the equity structure begins as:

Stakeholder	Shares (Illustrative, 10,000 total)	Ownership %
Owner / Founder	6,000	60.0%
Founding Team + Strategic Partners	2,000	20.0%
Investor Pool (reserved, unissued)	2,000	20.0%
Total	10,000	100.0%

The investor pool is a reservation, not issued equity. Until an investor closes, the effective ownership is Founder 75% / Founding Team 25% of issued shares.

4.2 Dilution Mechanics

Per params.md Section 8.2:

- The 20% investor pool is the **ceiling** for outside capital during the current planning phase
- If the investor requires more than 20%, dilution comes proportionally from the other two pools
- The founder maintains majority control (>50%) through Production in all modeled scenarios

Two dilution models are presented:

Model A — Investor buys from the reserved pool (no new shares issued): The investor purchases shares from the 20% reserved pool. Founder and founding team ownership percentages do not change. This is the simplest structure.

Model B — New shares issued (standard VC approach): New shares are issued to the investor, diluting all existing holders proportionally. This is the standard approach and is modeled in detail below.

4.3 Cap Table Evolution — Scenario S2 (Investor During Pilot 1-to-2 Transition)

S2 — Single Round at Post-Pilot 1 Valuation (\$450K midpoint)

Parameter	Value
Pre-money valuation	\$450,000
Investment amount	\$75,000-\$150,000
Post-money valuation	\$525,000-\$600,000
Investor ownership	14.3%-25.0%

Cap table after S2 investment (range):

Stakeholder	Pre-Investment	Post (\$75K raise)	Post (\$100K raise)	Post (\$150K raise)
Founder	60.0%	51.4%	49.0%	45.0%
Founding Team	20.0%	17.1%	16.3%	15.0%
Investor	0.0%	14.3%	16.3%	25.0%
Unallocated Pool	20.0%	17.1%	18.4%	15.0%
Total	100.0%	100.0%	100.0%	100.0%

Founder control analysis: At \$75K raise, founder retains 51.4% (majority control). At \$100K, founder drops to 49.0% — below the 50% threshold. At \$150K, founder holds 45.0%. To maintain >50% control at the \$450K valuation, the raise must stay at or below ~\$83K (18.5% dilution max at this valuation).

Recommendation: If raising at \$450K pre-money, cap the round at \$80K-\$90K to preserve founder majority. Alternatively, negotiate a higher pre-money valuation or use the reserved pool model (Model A) where founder ownership is unaffected.

S2 — Using the Reserved Pool (Model A, Recommended)

Stakeholder	Pre-Investment	Post (\$75K from pool)	Post (\$100K from pool)	Post (\$150K from pool)
Founder	60.0%	60.0%	60.0%	60.0%
Founding Team	20.0%	20.0%	20.0%	20.0%
Investor	0.0%	14.3%	16.3%	20.0%
Remaining Pool	20.0%	5.7%	3.7%	0.0%
Total	100.0%	100.0%	100.0%	100.0%

Under Model A, the founder always retains 60%. The trade-off is that the reserved pool shrinks, leaving less room for future investors or strategic hires.

4.4 Cap Table Evolution — Scenario S3 (Investor After Pilot 1)

S3 — Single Round at Post-Pilot 1 Valuation (\$450K midpoint)

Results are identical to S2 above, since the valuation is the same. The difference is timing: S3 investor arrives earlier (right after Pilot 1), giving the company more runway to execute Pilot 2.

S3 — Two-Round Scenario (Post-Pilot 1 + Pre-Production)

This models a company that raises a small round after Pilot 1 and a larger round before Production.

Parameter	Round 1 (Post-Pilot 1)	Round 2 (Pre-Production)
Pre-money valuation	\$450,000	\$2,000,000
Investment amount	\$75,000	\$200,000
Post-money valuation	\$525,000	\$2,200,000
Round dilution	14.3%	9.1%

Cap table evolution (Model B — new shares issued):

Stakeholder	Month 0	After Round 1	After Round 2
Founder	60.0%	51.4%	46.7%
Founding Team	20.0%	17.1%	15.6%
Investor (Round 1)	0.0%	14.3%	13.0%
Investor (Round 2)	0.0%	0.0%	9.1%

Unallocated Pool	20.0%	17.1%	15.6%
Total	100.0%	100.0%	100.0%

Founder control analysis: After two rounds, founder holds 46.7% — below 50%. To preserve majority through two rounds, the company should use Model A (reserved pool) for at least one of the rounds, or negotiate higher valuations.

Cap table evolution (Model A — reserved pool, both rounds):

Stakeholder	Month 0	After Round 1	After Round 2
Founder	60.0%	60.0%	60.0%
Founding Team	20.0%	20.0%	20.0%
Round 1 Investor	0.0%	14.3%	14.3%
Round 2 Investor	0.0%	0.0%	5.7%
Remaining Pool	20.0%	5.7%	0.0%
Total	100.0%	100.0%	100.0%

Under Model A, both rounds together consume the full 20% investor pool. Founder retains 60% throughout but has no remaining pool for future raises.

4.5 Dilution Summary Table — All Scenarios

Scenario Code	Raise Amount	Pre-Money	Post-Money	Investor %	Founder % (Model A)	Founder % (Model B)	Founder >50%? (Model A)	Founder >50%? (Model B)
S1 (all levels)	\$0	N/A	N/A	0%	60.0%	60.0%	Yes	Yes
S2 at \$450K, \$75K raise	\$75K	\$450K	\$525K	14.3%	60.0%	51.4%	Yes	Yes
S2 at \$450K, \$100K raise	\$100K	\$450K	\$550K	18.2%	60.0%	49.1%	Yes	No
S2 at \$450K, \$150K raise	\$150K	\$450K	\$600K	25.0%	60.0%	45.0%	Yes	No
S3 at \$450K, \$75K raise	\$75K	\$450K	\$525K	14.3%	60.0%	51.4%	Yes	Yes
S3 at \$1.15M, \$150K raise	\$150K	\$1.15M	\$1.30M	11.5%	60.0%	53.1%	Yes	Yes
S3 at \$1.15M, \$250K raise	\$250K	\$1.15M	\$1.40M	17.9%	60.0%	49.3%	Yes	No
S3 at \$2.25M, \$375K raise	\$375K	\$2.25M	\$2.63M	14.3%	60.0%	51.4%	Yes	Yes
S3 at \$2.25M, \$500K raise	\$500K	\$2.25M	\$2.75M	18.2%	60.0%	49.1%	Yes	No
Two-round: \$75K + \$200K	\$275K	\$450K / \$2.0M	—	22.1% combined	60.0%	46.7%	Yes	No

4.6 Founder Control Preservation Rules

Based on the analysis above, the following rules preserve founder majority (>50%) in all scenarios:

Rule	Description
Rule 1: Use Model A whenever possible	Allocating from the reserved pool preserves founder at 60% regardless of round size (up to the 20% pool ceiling)
Rule 2: If using Model B, cap dilution at 16% per round	At any valuation, limiting investor equity to 16% keeps founder above 50.4%
Rule 3: If raising >\$100K at Post-Pilot 1	Negotiate pre-money above \$500K, or use Model A
Rule 4: For two-round scenarios	Use Model A for at least the first round; Model B is acceptable for the second if valuation is \$1.5M+
Rule 5: Super-voting shares	As a structural alternative, founder can hold 10:1 super-voting shares to maintain control regardless of economic dilution

5. Investor Return Modeling

5.1 Return Framework

Investors in early-stage East African health tech typically target 5-10x returns over a 5-7 year horizon. The following models show what different return multiples look like for investors at each valuation stage.

5.2 Return at Post-Pilot 1 Entry (\$450K Pre-Money, \$75K Investment for 14.3%)

Return Multiple	Required Exit Valuation	Investor Proceeds	Timeline Assumption
2x	\$1.05M	\$150K	2-3 years
5x	\$2.63M	\$375K	3-5 years
10x	\$5.25M	\$750K	5-7 years
20x	\$10.5M	\$1.5M	7-10 years

5.3 Return at Post-Pilot 2 Entry (\$1.15M Pre-Money, \$150K Investment for 11.5%)

Return Multiple	Required Exit Valuation	Investor Proceeds	Timeline Assumption
2x	\$2.60M	\$300K	2-3 years
5x	\$6.50M	\$750K	3-5 years
10x	\$13.0M	\$1.5M	5-7 years
20x	\$26.0M	\$3.0M	7-10 years

5.4 Return at Pre-Production Entry (\$2.25M Pre-Money, \$375K Investment for 14.3%)

Return Multiple	Required Exit Valuation	Investor Proceeds	Timeline Assumption
2x	\$5.25M	\$750K	2-3 years
5x	\$13.1M	\$1.88M	3-5 years
10x	\$26.3M	\$3.75M	5-7 years
20x	\$52.5M	\$7.5M	7-10 years

5.5 Required Exit Valuation Plausibility Check

Exit Valuation	Revenue Multiple Required (at 6x)	Implied Annual Revenue	Plausibility
\$2.6M	6x	\$433K/year (\$36K/mo)	Achievable by Production Year 2
\$5.25M	6x	\$875K/year (\$73K/mo)	Achievable by Production Year 3 with Kenya expansion
\$13M	6x	\$2.2M/year (\$183K/mo)	Requires multi-country scale + facility SaaS tier
\$26M	6x	\$4.3M/year (\$361K/mo)	Requires dominant market position in 2+ countries
\$52.5M	6x	\$8.75M/year (\$729K/mo)	Requires regional scale (3+ countries) + premium products

5.6 Revenue Multiples for East African Health Tech

Multiple Range	Company Stage	Applicability to Health Hub
3-5x revenue	Profitable, growing	Post-Production Year 3+
5-8x revenue	High-growth, not yet profitable	Production Year 1-3
8-12x revenue	Category leader, rapid expansion	Dominant in 2+ markets
12-20x revenue	Breakout company, Series B+	Regional platform with network effects

5.7 Investor Return by Scenario Code

For the most common investor entry points per params.md:

Scenario	Entry Valuation	Investment	Equity %	5x Exit Value	10x Exit Value	IRR at 5x (5yr)	IRR at 10x (5yr)
S2-O2-I2 (base)	\$450K	\$100K	18.2%	\$2.75M	\$5.50M	38%	58%
S2-O1-I1 (lean)	\$300K	\$50K	14.3%	\$1.75M	\$3.50M	42%	63%
S2-O3-I3 (full)	\$600K	\$150K	20.0%	\$3.75M	\$7.50M	36%	55%
S3-O2-I2 (base)	\$1.15M	\$150K	11.5%	\$6.50M	\$13.0M	35%	53%
S3-O3-I3 (full)	\$2.25M	\$375K	14.3%	\$13.1M	\$26.3M	34%	52%

IRR calculations assume a 5-year hold period and single exit event.

6. Comparable Deals

6.1 East African Health Tech Funding Rounds (2020-2025)

Company	Country	Round	Year	Amount Raised	Valuation (Est.)	Stage at Raise	Relevance to Health Hub

mPharma	Ghana/Kenya	Series D	2022	\$35M	~\$200M	Growth; 300+ pharmacies	Pharmacy marketplace; validates sector valuations at scale
Helium Health	Nigeria/Kenya	Series B	2022	\$30M	~\$150M	Growth; 10,000+ providers	EHR + telemedicine; multi-country
Zuri Health	Kenya	Seed	2022	\$1.3M	~\$5-8M (est.)	Early traction; telemedicine app	Closest comparable — mobile-first telehealth in East Africa
Ilara Health	Kenya	Series A	2022	\$10.5M	~\$30-40M (est.)	Scaling; diagnostic devices	Diagnostics marketplace; partner model similar to Health Hub
Rocket Health	Uganda	Seed	2021	\$5M	~\$15-20M (est.)	Early traction; teleconsultation + pharmacy delivery	Telehealth + pharmacy in East Africa; admin-assisted model
Damu Health	Ethiopia	Pre-Seed	2023	\$250K	~\$1-2M (est.)	Pre-revenue; health records platform	Ethiopian health tech; very early stage
Chapa (fintech/health)	Ethiopia	Seed	2023	\$1.5M	~\$6-8M (est.)	Early revenue; payment infra	Ethiopian tech startup benchmark (not pure health)
MyDawa	Kenya	Series A	2021	\$4M	~\$12-15M (est.)	Growing; online pharmacy	Pharmacy platform; validates East African health e-commerce

6.2 Implied Valuation Benchmarks

Stage	Comparable Range	Health Hub Proposed	Position vs Comps	Justification
Pre-Seed / Post-Pilot 1	\$500K-\$2M	\$300K-\$600K	Below average	Conservative; reflects part-time team and Ethiopia-first (lower GDP market)
Seed / Post-Pilot 2	\$2M-\$8M	\$800K-\$1.5M	Below average	Intentionally discounted; validates model before pricing at market
Pre-Series A / Pre-Production	\$5M-\$15M	\$1.5M-\$3M	Below average	Leaves significant upside for investor; de-risks entry

6.3 Why Health Hub Values Below Comparables

Factor	Impact on Valuation	Mitigation Path
Part-time team	-20 to -30% vs full-time comparable	Convert key members to full-time post-investment
Ethiopia-first (lower GDP)	-10 to -20% vs Kenya-first	Kenya expansion in Pilot 2 addresses this
No previous founder exits	-10 to -15%	Track record built through successful pilots
Pre-revenue at first raise	-15 to -25%	Revenue begins in Pilot 1; grows through Pilot 2
Nascent regulatory environment	-5 to -10%	First-mover advantage in unregulated market

6.4 Valuation Floor — Asset-Based Minimum

Even ignoring future cash flows, Health Hub has a defensible asset floor:

Asset	Value
Codebase (replacement cost per params.md Section 16)	\$33,500-\$45,500
Business plan and documentation suite	\$5,000-\$8,000
Team relationships and domain expertise (12 contributors)	\$15,000-\$25,000 (recruitment cost equivalent)
Partner network and market intelligence (Ethiopia)	\$5,000-\$10,000
Total asset floor	\$58,500-\$88,500

The asset floor confirms that even the lowest proposed valuation (\$300K) represents a 3.4-5.1x premium over replacement cost — typical for a technology company with assembled team and market knowledge.

7. Key Takeaways

#	Finding	Implication
1	Scorecard and Berkus methods converge on \$400K-\$1M for Post-Pilot 1	The proposed \$300K-\$600K range is conservative and investor-friendly
2	Model A (reserved pool) preserves founder at 60% in all scenarios	Strongly recommended for first raise; simplifies negotiation
3	Model B (new shares) risks dropping founder below 50% if raise exceeds ~\$83K at \$450K valuation	If Model B is required by investor, negotiate higher pre-money or use super-voting shares
4	Comparable East African deals support \$500K-\$2M at seed stage	Health Hub's proposed range is below comps, giving investors a value entry point
5	5x investor return requires exit at \$2.6M-\$13M depending on entry	All achievable within 5-7 years based on revenue projections in the business plan

6	Two-round strategy (Post-Pilot 1 + Pre-Production) fully consumes the 20% pool	If more capital is needed, founder must accept additional dilution beyond 20% or find non-dilutive capital
7	DCF method becomes reliable only at Pre-Production when revenue data exists	Do not lead with DCF in early-stage negotiations; use it for sanity checks

8. Cross-References

Document	Relevance
params.md	Equity structure (Section 8), revenue model (Section 11), phase definitions (Section 13), scenario matrix (Section 14-15)
03-cost-breakdown.md	Total capital requirements by scenario — inputs to post-money calculation
05-revenue-modelling.md	Revenue projections used in DCF and exit valuation plausibility
12-fundraising-readiness.md	Investor materials, deal structures, and term sheet guidance
15-competitive-analysis.md	Competitor landscape supporting comparable transactions analysis

End of Document 11 — Valuation & Dilution. All figures trace to params.md assumptions.